

**Prof Jeffrey S Morris** @jsm2334Jul 7, 2026 · 8 tweets · [jsm2334/status/2074322827759497323](https://twitter.com/jsm2334/status/2074322827759497323)

It has long been known that SIDS follows a characteristic background age distribution: risk rises over the first couple of months to a mode at 2 months, a median around 3 months, then declines sharply through 12 months.

SIDS wasn't formally defined until 1969 — but this same pattern shows up in related infant-death categories going much further back. Abramson (1944) presented a table with the age distribution of accidental mechanical suffocation in infancy in New York from 1934–1943, well before any routine infant vaccination. This table displays this characteristic pattern.

To see how stable this pattern is, I pulled the data from the Abramson table and digitized the figures from three later SIDS studies, then normalized each to an annual age distribution summing to 100% to put them on a common scale:

1965–1968, USA (Washington)

1993–1996, UK

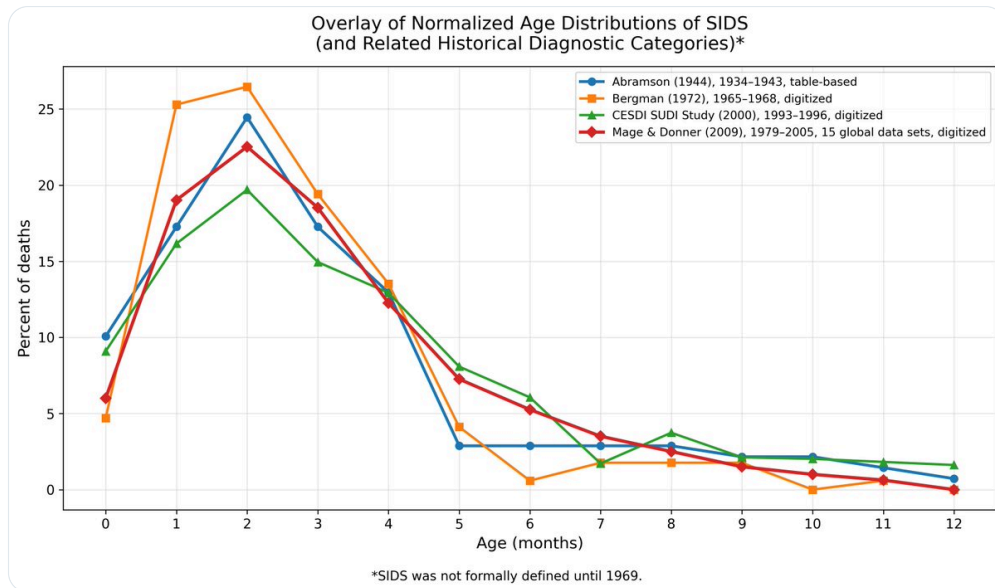
1979–2005, pooled across 15 global datasets

Overlaying all four (plot below), the age distribution has stayed remarkably consistent across many decades and completely different vaccination eras — from before routine infant vaccines existed, in early days of vaccination, through the modern schedule.

That's rather inconvenient for the claim that the SIDS peak at 2–4 months must be vaccine-caused simply because vaccines are given around that age. The peak predates the vaccines and persists unchanged regardless of them.

It also drives home why adjusting for age confounding is essential in any analysis of whether SIDS risk rises after vaccination. With a background curve this steep, a method that doesn't adjust for age will conclude "vaccine harm" no matter what — even when there is no vaccine effect at all. That's exactly what my simulation studies found: a 100% false-positive rate under the true null when age went unadjusted.

Full links to every paper, the source tables and figures, the extracted data, the normalized distributions, and everything needed to reproduce these plots are in the posts below and in the GitHub repo linked at the end of the thread.



Here is the first paper, Abramson (1944): [https://www.jpeds.com/article/S0022-3476\(44\)80005-1/abstract](https://www.jpeds.com/article/S0022-3476(44)80005-1/abstract)

It summarizes 139 cases of "accidental mechanical suffocation deaths in infancy." Table III gives the age distribution of these cases, which I used to generate the curve in the plot. For the 21 cases the table groups into the 6–12 month range, I distributed them across the individual months as 4, 4, 4, 3, 3, 2, and 1, respectively.

This paper wasn't modeling SIDS as such — the category and definition did not exist yet.

But it was the first in a series of papers that led to the recognition of a distinct category of unexplained infant crib death, work that culminated in the formal definition of SIDS in 1969. I'll trace that lineage in the next post.

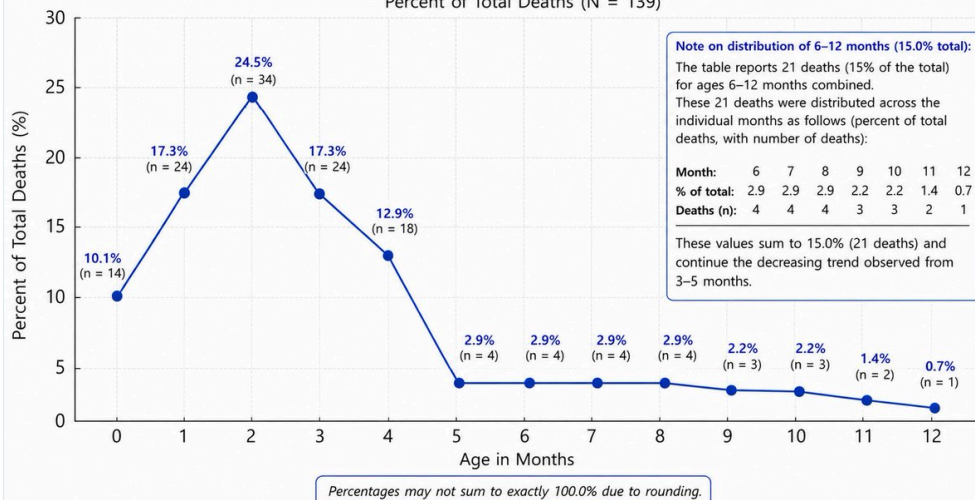
TABLE III

DEATHS FROM ACCIDENTAL MECHANICAL SUFFOCATION IN 139 INFANTS UNDER 1 YEAR (ACCORDING TO AGE AND SEX)

AGE (MO.)	SEX	NUMBER	PER CENT
0 to 1	Total	14	10.0
	M	8	6.0
	F	6	4.0
1 to 2	Total	24	17.0
	M	14	10.0
	F	10	7.0
2 to 3	Total	34	25.0
	M	26	19.0
	F	8	6.0
3 to 4	Total	24	17.0
	M	17	12.0
	F	7	5.0
4 to 5	Total	18	13.0
	M	16	12.0
	F	2	1.0
5 to 6	Total	4	3.0
	M	1	1.0
	F	3	2.0
6 to 12	Total	21	15.0
	M	13	9.0
	F	8	6.0
Total		139	100.0
M		95	69.0
F		44	31.0

Distribution of Deaths from Accidental Mechanical Suffocation in Infants Under 1 Year by Age in Months, 1934–1943, Abramson (1944)

Percent of Total Deaths (N = 139)



The age distribution on the previous page was of deaths the authors classified as "accidental mechanical suffocation deaths in infancy." But that classification was itself contested from early on.

Wooley (1945)

<https://www.sciencedirect.com/science/article/abs/pii/S0022347645800859> responded directly to Abramson, pointing out that suffocation was presumed, not confirmed — many of

these infants had simply died in their sleep with no explanation and were assumed to have suffocated. It was one of the earliest explicit arguments that infant deaths certified as mechanical suffocation likely belonged to a broader category of sudden unexplained infant death, and that attributing them all to suffocation was probably wrong.

The rest of the lineage followed:

Werne & Garrow (1953) <https://pmc.ncbi.nlm.nih.gov/articles/PMC1937460/> published one of the first systematic pathological studies of infants found dead unexpectedly, emphasizing that many deaths remained unexplained even after autopsy.

Adelson & Kinney (1956) pubmed.ncbi.nlm.nih.gov/13322513/ developed the concept of "sudden and unexpected death in infancy," explicitly separating these deaths from obvious accidental suffocation (see quote below)

At the 1969 Seattle conference, Beckwith proposed the term Sudden Infant Death Syndrome – the first formal definition of the syndrome, later set out in the peer-reviewed Beckwith (1973): pubmed.ncbi.nlm.nih.gov/4351768/

So the Abramson (1944) age distribution is not irrelevant to SIDS. Contemporaries recognized at the time that many of these "suffocation" deaths were the very same sudden, unexplained infant deaths that would later be named SIDS – which is why the curve looks the way it does, and why it belongs in this overlay.

THE RISING TIDE OF NATURAL EXPLANATIONS

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The lack of satisfactory objective evidence concerning the cause of these deaths in infancy and childhood accounts for the frequency with which such speculative explanations as suffocation by bedding, lymphatism, or status thymicolymphaticus are proposed. Although the importance of an infectious process in the causation of these deaths has been emphasized in recent writings, there is no agreement as to the types of infective agents involved or the lesions produced.

Drs. Adelson and Kinney (18)

The next study, Bergman (1972): pubmed.ncbi.nlm.nih.gov/5041320/, presented a histogram of SIDS cases from Washington State, USA, between January 1, 1965 and September 1, 1968.

The values from the histogram were digitized, normalized, and the resulting age distribution plotted.

Below is the original histogram alongside the age-distribution curve we generated from it.

The specifics of how the data were digitized and normalized:

1. The two neonatal bars were read as approximately 2 and 6 cases and combined into month 0.
2. The monthly bars were read as: month 1 = 43, month 2 = 45, month 3 = 33, month 4 = 23, month 5 = 7, month 6 = 1.
3. The broad bar spanning roughly months 7–10 was treated as a monthly height of about 3 cases for months 7, 8, and 9.
4. Month 11 was read as about 1 case; months 10 and 12 as 0.
5. These counts summed to 173, close enough to the actual N=170 SIDS cases in the paper (within 1.7%).

6. The extracted counts were then normalized to sum to 100%, and the resulting distribution was plotted.

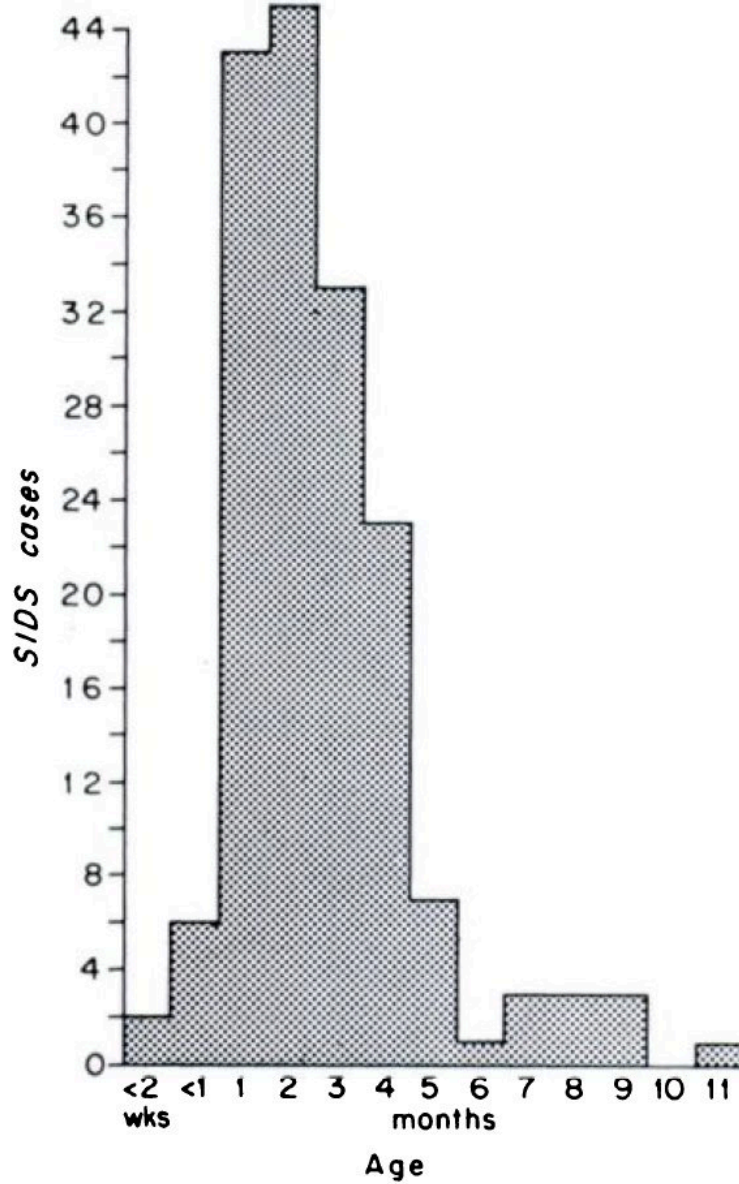
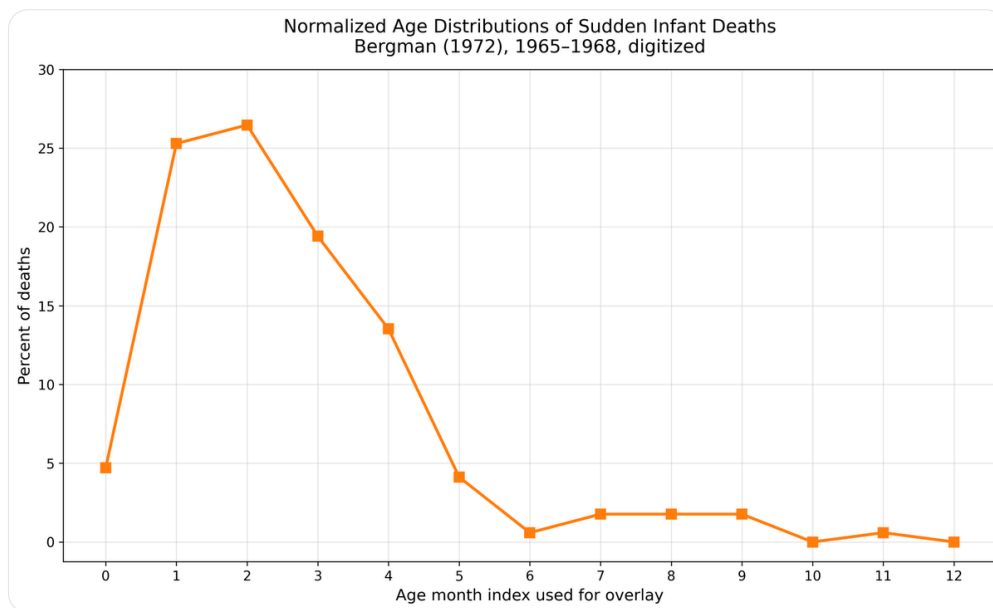


FIG. 1. The age distribution of 1970 cases of SIDS, January 1, 1965-September 1, 1968.



The next study is the CESDI SUDI data, plotting SIDS deaths by age in weeks from 1993–1996, presented as Figure 15.2 (p. 330) in Duncan and Byard (2018), *SIDS: Sudden Infant and Early Childhood Death: The Past, the Present and the Future*:

<https://www.ncbi.nlm.nih.gov/books/NBK513384/>

1. We used the yellow SIDS curve only.
2. Approximate percentages were read off the y-axis in 4-week bins: 1–4w = 9.0, 5–8w = 16.0, 9–12w = 19.5, 13–16w = 14.8, 17–20w = 12.8, 21–24w = 8.0, 25–28w = 6.0, 29–32w = 1.7, 33–36w = 3.7, 37–40w = 2.1, 41–44w = 2.0, 45–48w = 1.8, 49–52w = 1.6.
3. These were mapped to month indices 0–12, close enough to not significantly affect the overall shape comparison.
4. We then renormalized to sum to 100%, and plotted as the age-distribution curve shown.

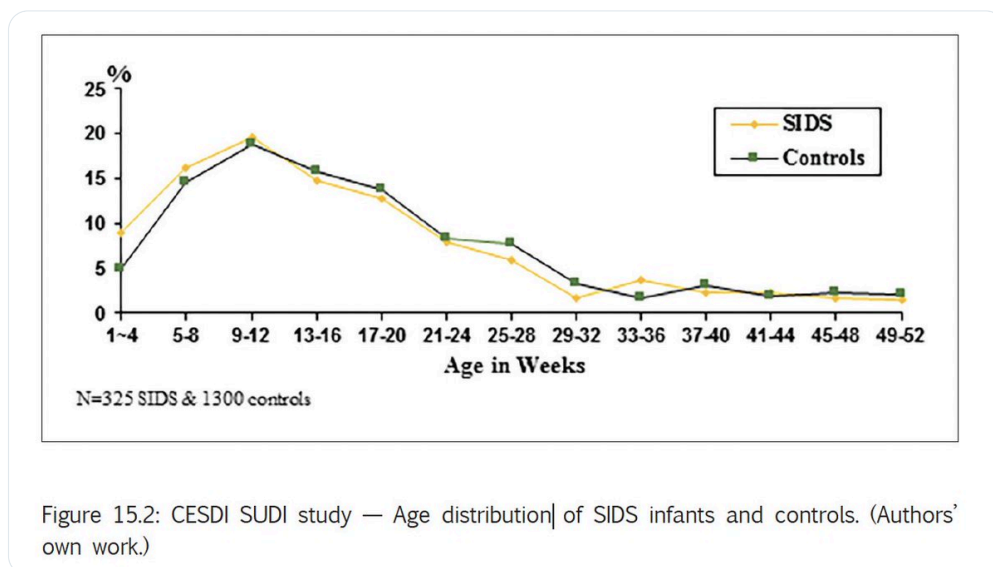
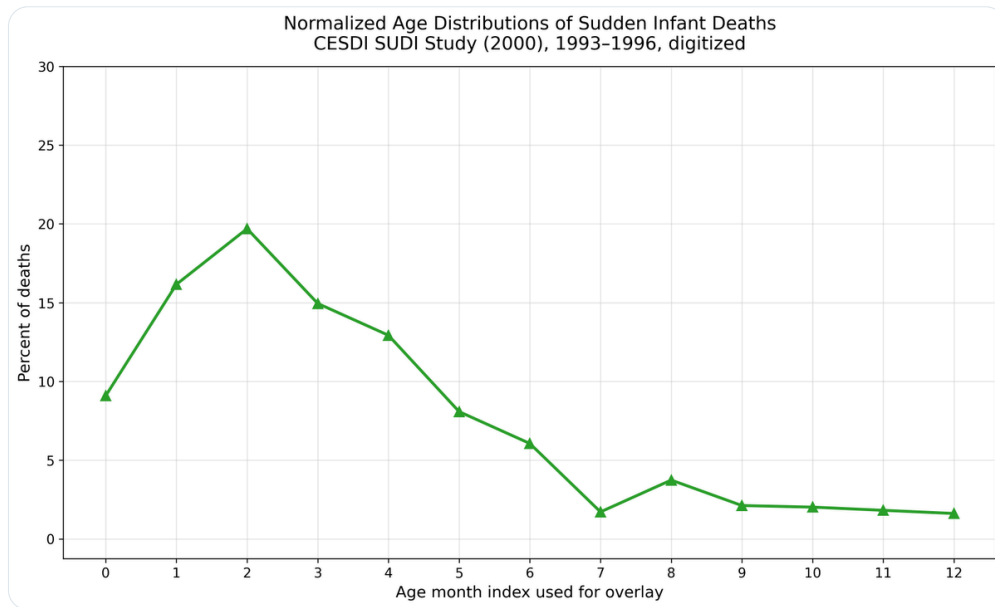


Figure 15.2: CESDI SUDI study — Age distribution of SIDS infants and controls. (Authors' own work.)



The fourth study is a pooled set of 19,949 SIDS cases drawn from 15 studies worldwide, spanning 1979–2005, published in Mage & Donner (2009):

<https://pmc.ncbi.nlm.nih.gov/articles/PMC2798085/>

Figure 2 (below) presents a bar plot of monthly SIDS counts aggregated across all the studies (shown below), which we digitized, normalized, and plotted.

1. Monthly bar heights were read approximately from Figure 2 against the 0–5000 y-axis grid.
2. The extracted counts were: 1200, 3800, 4500, 3700, 2450, 1450, 1050, 700, 500, 300, 200, 125, 0.
3. These were normalized to sum to 100% to yield the age distribution plotted below.

Note that the extracted counts sum to 19,975 — within 26 cases (about 0.1%) of the published total of 19,949 — confirming the digitization closely reproduces the underlying data.

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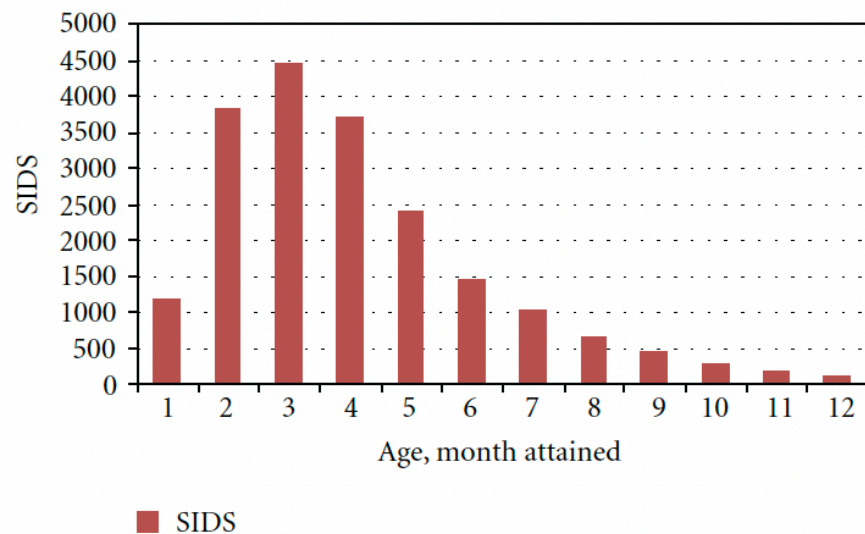
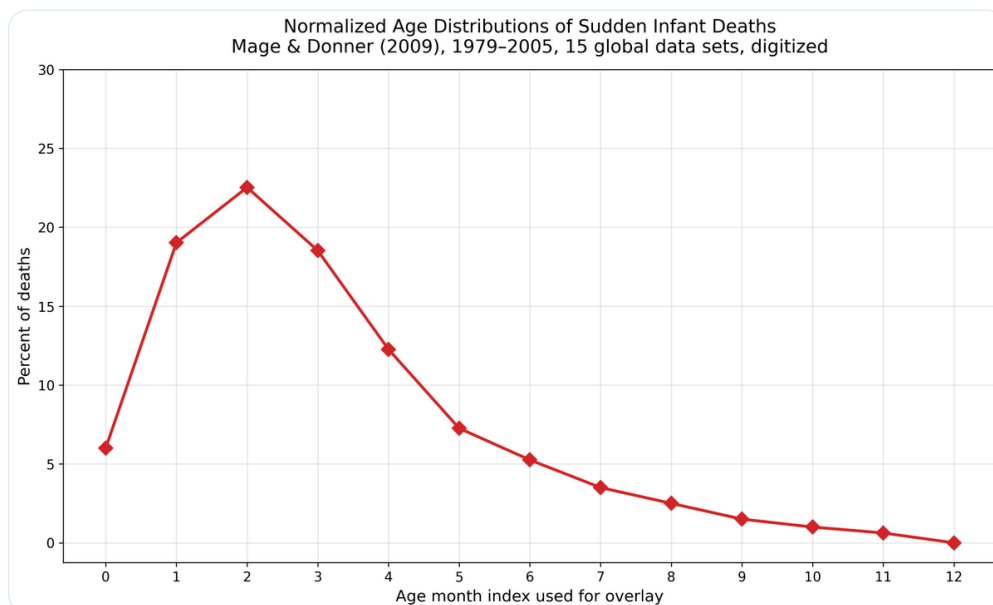


FIGURE 2: Age distribution from 15 global data sets combined versus month attained of 19,949 SIDS [21].

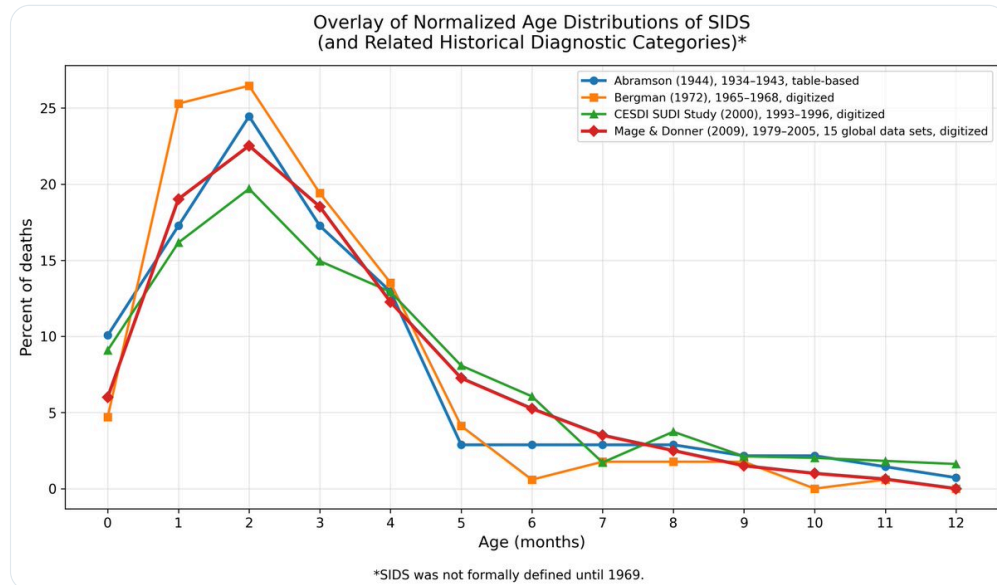


All four of these age distributions were normalized to sum to 100%, so by overlaying them on top of each other, we get a reasonable comparison of the underlying shapes of the age distributions (even with minor differences in how the data were partitioned into monthly bins)

We can see over the decades, the shape of the age distribution of SIDS (and precursors) has

remained remarkably consistent, increasing from birth up to a mode at 2m, a median about 3m, with a sharp decline through 12m.

This github repository (<https://github.com/Jeffrey-S-Morris/SIDS-SCCS-Simulation/tree/main>) has all plots, links to papers, and the .xls file of data extracted from the tables and figures and the corresponding age distributions to reproduce all of the results in the overlaid plot.



Of course, I reiterate that it is not possible to make inference on causal vaccine effects from simple plots of SIDS by age.

This thread is simply showing the remarkable consistency of this canonical age distribution for unexplained infant crib deaths over time.

This thread is for those who DO try to make such inference, claiming the mode at 2m MUST be caused by vaccines simply because many infants receive vaccines at that time – this shows the shape predates vaccines and when there were far fewer vaccines so is not simply caused by vaccines.

This is also for those that suggest that modeling background age distribution to adjust for age confounding (as in the published modified SCCS and in mine and Jikky's simulation studies) is somehow trying to "hide vaccine harm"

When in fact the shape of this age distributions produces 100% false positive rate for interval based methods of the SCCS variety when there is in fact no vaccine effect, as demonstrated in my simulations x.com/jsm2334/status... (and Jikky's once the null simulations are added in x.com/jsm2334/status...). This age confounding is essential and must be included in any such analysis.

This all (including the simulations) strongly supports the idea that modified SCCS with age bins based on this canonical longnormal style age distribution is an excellent method, perhaps the best available method, to analyze existing data to investigate the question of whether there is increased risk of SiDS (or other related unexplained infant crib deaths) immediately after vaccination or not, such as the six studies cited here and

(x.com/jsm2334/status...), and with my simulation studies showing clear evidence the method can find vaccine harm when it is there (x.com/jsm2334/status...)

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